# Weekly cetuximab + irinotecan

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Day 1 · chemotherapy and antibody

**Active medicines:** Cetuximab + Irinotecan

**Emetogenic risk:** Moderate

| Medicine | Dose / route | When |
|---|---|---|
| Chlorphenamine | 10 mg intravenous (IV) bolus | At least one hour before cetuximab<br>Repeat for subsequent cetuximab infusions; assess prior infusion reactions. |
| Dexamethasone | 8 mg IV bolus | At least one hour before cetuximab on Day 1<br>This corticosteroid also covers the treatment-day corticosteroid part of antiemesis where appropriate. Pharmacy must reconcile route, dose and timing; do not automatically add a second 8 mg antiemetic dose. |
| Ondansetron | 16 mg PO once | Before Day 1 chemotherapy<br>NCIS V8 moderate-risk row; consider QT risk, low electrolytes, interactions and the individual prescription. |
| Dexamethasone | 8 mg PO daily | Days 2 and 3 after the administered Day 1 chemotherapy<br>NCIS V8 delayed prophylaxis. Reconcile with any other steroid prescription; re-anchor dates if treatment is delayed. |
| Metoclopramide | 10 mg PO up to three times daily as required | Rescue nausea treatment only as prescribed<br>NCIS V8 rescue option; check contraindications, renal function, interactions and prescribed duration. |
| Atropine sulphate | 250 micrograms subcutaneously | Before irinotecan when prescribed; assess acute cholinergic symptoms<br>Avoid in glaucoma. Use with caution in asthma; review suitability before prescribing. This treats/prevents the acute cholinergic syndrome; it is not the delayed-diarrhoea rescue medicine. |

### Additional phase-specific support

- Antiemetic classification refers to the active treatment day. Reconcile duplicate steroid entries before prescribing.
- No automatic granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) schedule is added; use the patient's neutropenia history, risk and consultant plan.
- Cetuximab premedication and infusion observation remain necessary even on an antibody-only day.
- Start local EGFR skin care; monitor magnesium, calcium and potassium before and periodically during therapy, then monthly for two months after the final dose.
- Provide a written irinotecan delayed-diarrhoea rescue and escalation plan before treatment; distinguish it from immediate infusion-related cramps/sweating. Fever, severe pain, dehydration or diarrhoea not responding to the prescribed plan needs urgent review.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00330 v6 · p4 · NCCP antiemetics v8, Table 1 pp4–5